AIMLEI 2026

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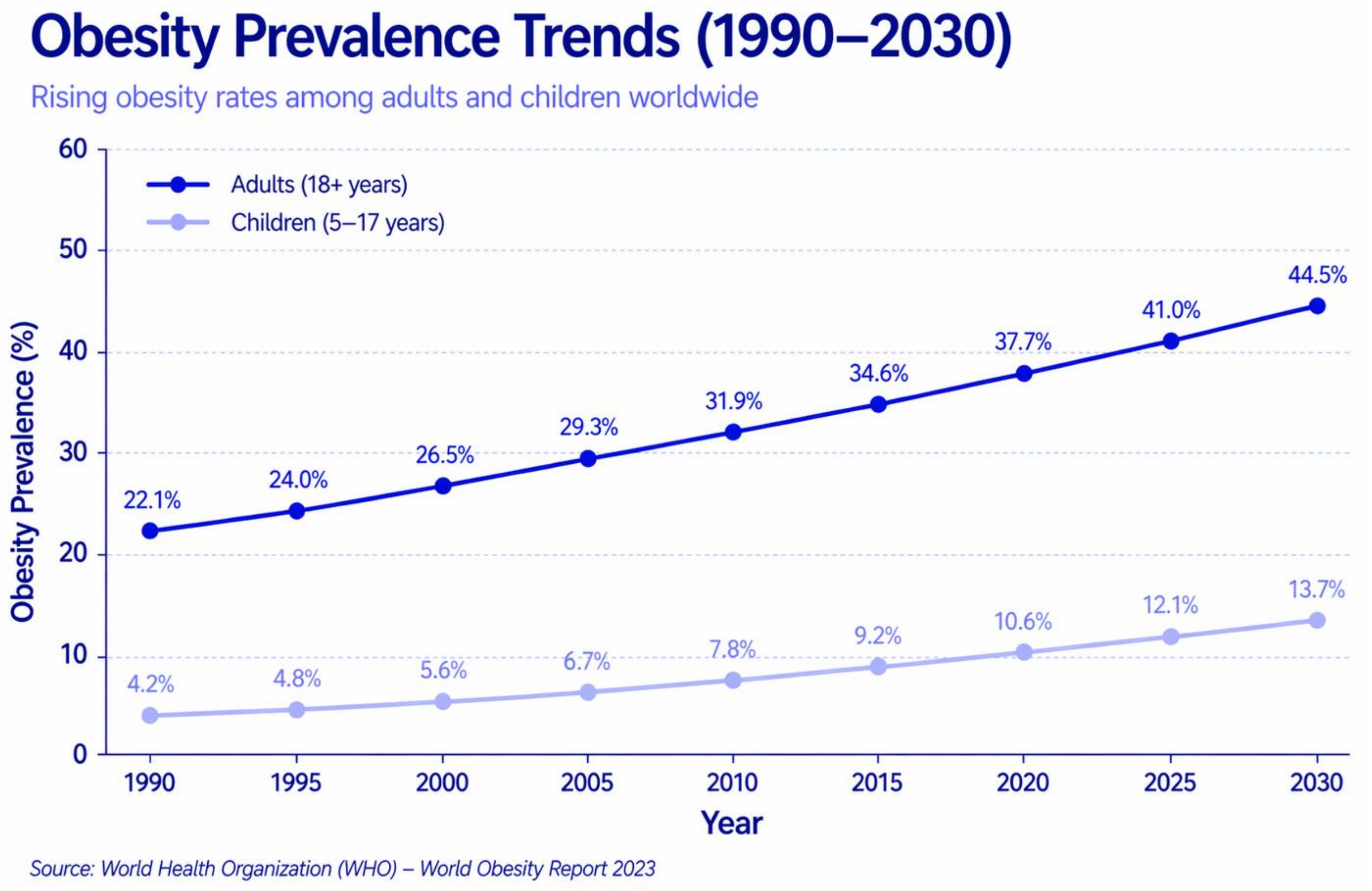
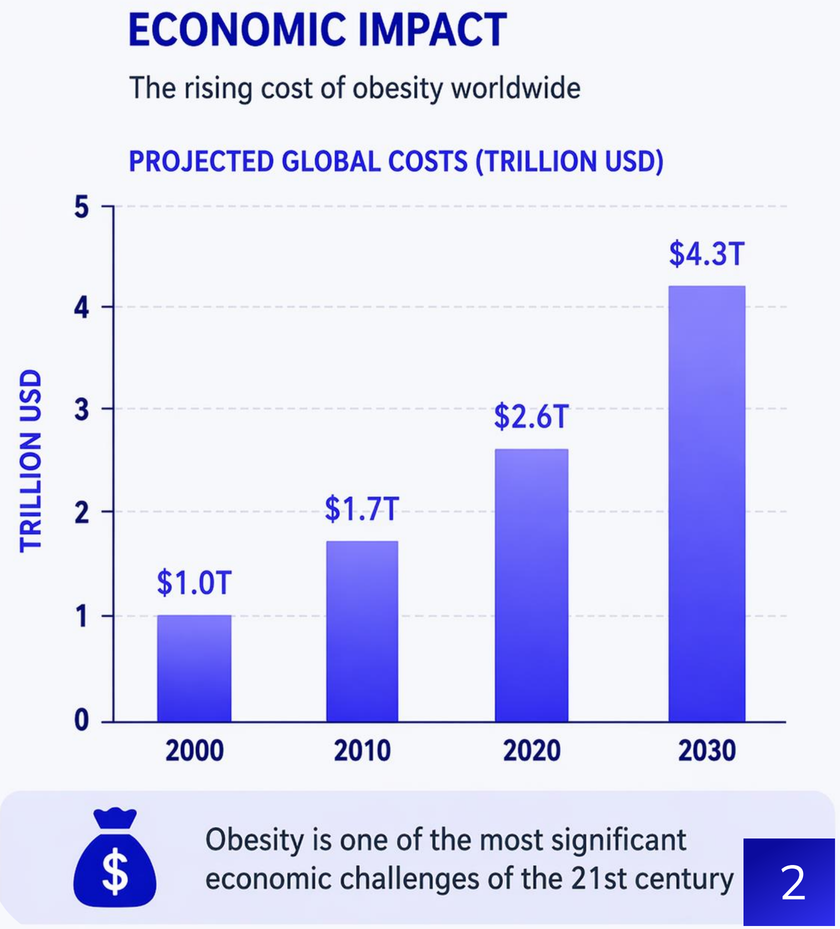
Healora:

From Obesity to Longevity through a Digital Twin

Skoltech 15 Years of Science and Technologies

A 40-year trend: obesity doubles, health spending as % of GDP declines – a growing public health crisis





Obesity care lacks scalable behavioral support: 70–80% weight regain within 1–2 years

Global

16%
obese

43%
overweight
(~3.3B adults)

In Russia

≥50%
overweight
doubled since 1990

Key Gaps

× One-size-fits-all advice – no real-life personalization
× No continuous, scalable behavioral support
× Drugs/surgery are costly and don't change habits

Majority have

≥2 cardiometabolic diseases

T2DM, hypertension, CVD, dyslipidemia

Proven but not scalable

5–10%
weight loss

58%
diabetes risk reduction

78%
lose weight

65%
maintain at 1 year **via habit change**

Without an engaging, personalized, scalable behavioral engine – clinical guidelines fail in real life

1. Review of current guidelines for the treatment of obesity, 2022.2 |
3. Diabetes Prevention Program (DPP): lifestyle change program (5–7% weight loss - 58% diabetes risk reduction).3,6,21
2. Lifestyle interventions for the management of overweight and obesity in adults, 2025.23 |
4. Look AHEAD trial: intensive lifestyle intervention and long-term weight loss / cardiometabolic improvements.13

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Scientific basis, open data, and validation plan for Healora Digital Twin

Scientific Evidence

- **Overweight/obesity –** higher resting heart rate, reduced HRV (autonomic imbalance), slower heart-rate recovery after exercise
  Strüven et al., 2021; Rossi et al., 2015; Dimkpa & Oji, 2010
- **30-sec sit-to-stand test** is reproducible, safe in obesity and cardiovascular risk
  Lázaro-Martínez et al., 2022

Training and validation pathway

Public data
NHANES

body measures, pulse, blood pressure, obesity labels

Multimodal obesity data
AI4FoodDB

100 overweight/obese Participants, wearables, biomarkers

Feature engineering baseline ML
Inputs

HRrest, HRpeak, HRR60, HRR120, BMI, waist, BP

Model goal

obesity-related cardiometabolic/autonomic risk score

Lab pilot
20-40 participants across BMI groups with wearable + sit-to-stand / step test

Digital Twin update
Use score as one node in Healora; combine later with mood, diet, sleep

8 habits on the path to longevity

Habit	Method	Expected Change	Time to Result	Evidence
Late-night eating	Micro-quest: no food after 9 PM	↓ 50% episodes	2–4 weeks	Healora POC
Meal logging	Gamified streaks	71% adherence	1 week	Healora POC
Daily steps	Step reminders + tracking	+2,000 steps/day	4 weeks	DPP trial
Sleep duration	Wind-down reminders	+1 hour/night	4 weeks	Clinical guidelines
HRV / stress	Breathing exercises	+10–20% HRV	8 weeks	Causal model
Diet (carbs vs fat)	Genotype-informed education	60%+ alignment	4 weeks	DIETFITS 2025
Medication adherence	Daily check-ins	>90% adherence	1 week	Post-market
Emotional eating	Mood logging before meals	↓ 30% episodes	4 weeks	Mental health

Healora

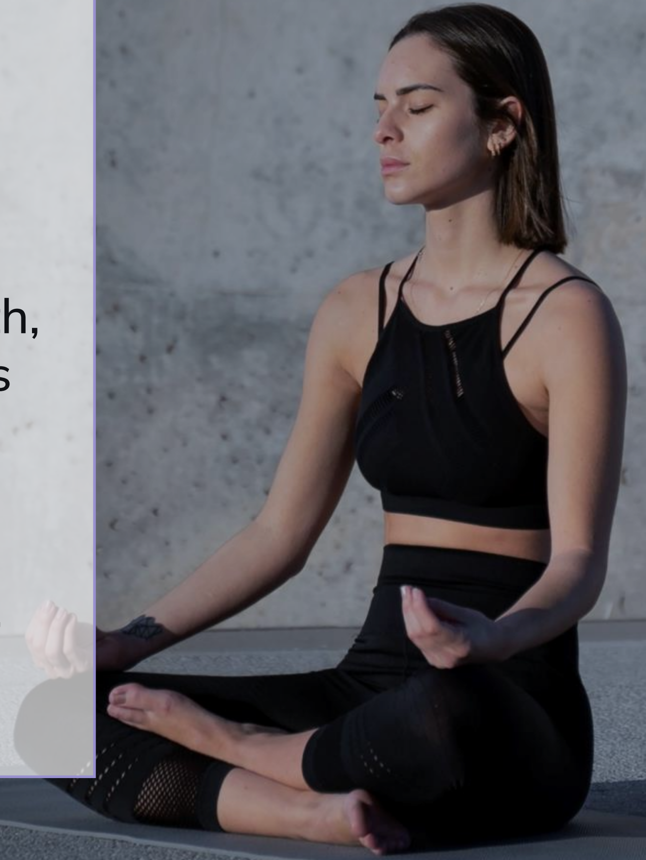
AI-powered Digital Twin for obesity prediction and prevention

Core concept

Healora combines two breakthrough technologies

» **Digital Twin**
a living AI model of your health, updated from 6+ data sources

» **Longevity Path**
an adaptive roadmap of daily micro-quests, medical checks, and education



Data sources

Blood	Clinical history
Diet logs	Wearables (HRV, sleep)
Genetics	Mental health

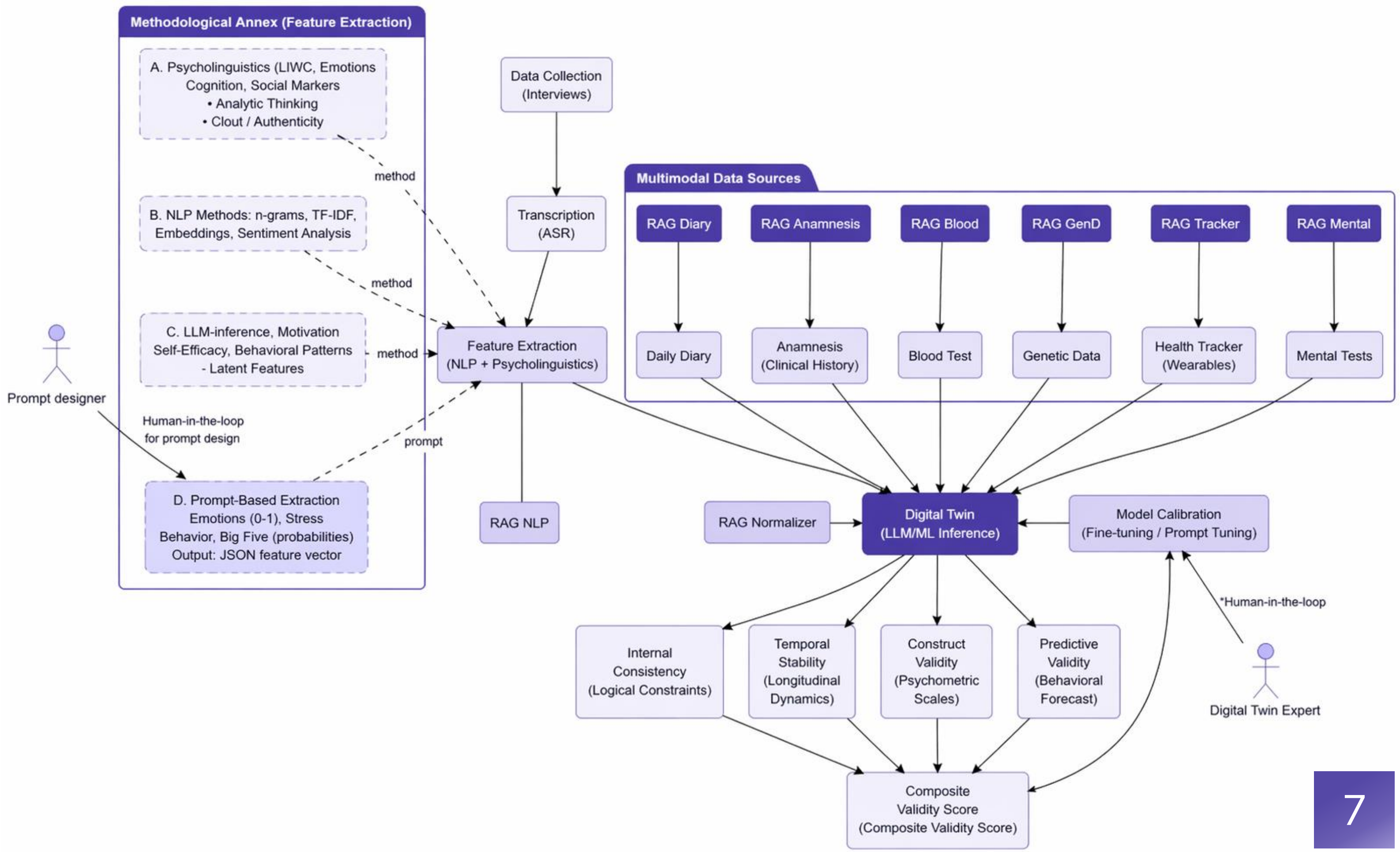
Healora's AI-powered Digital Twin predicts obesity risk and delivers a personalized Longevity Path

OMNI-CAS data

» **Polygenic Risk Scoring**
PRS analysis, PGS Catalog, heritability, health traits, disease predisposition

» **Validation & Benchmarking**
OpenHumans data, cohorts, accuracy, error analysis, prediction verification

» **Automated Data Ingestion**
Parsing, raw genetic files, standardization, cleaning, multi-source processing



See how Healora turns blood, genetics, and wearables into daily action

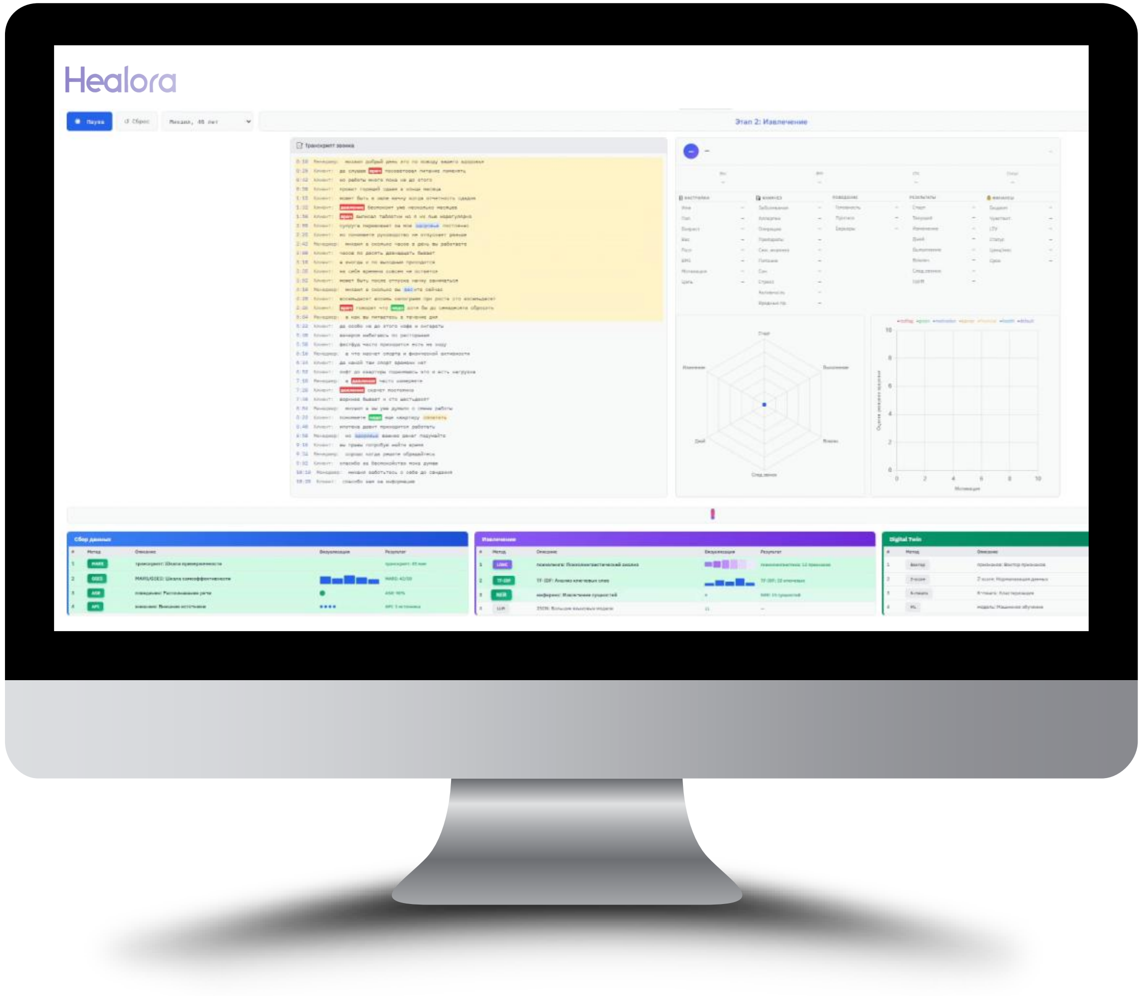
The Vision

» **Digital Twin**
a single, living model of the user,
fed by multimodal data (blood, genetics, wearables, mental health, diary)

» **Longevity Path**
an adaptive, gamified roadmap that turns clinical insights into daily micro-quests

» **Proven engagement**
Ritm POC showed 71% logging adherence vs 38% control (non-obvious, published-grade result)

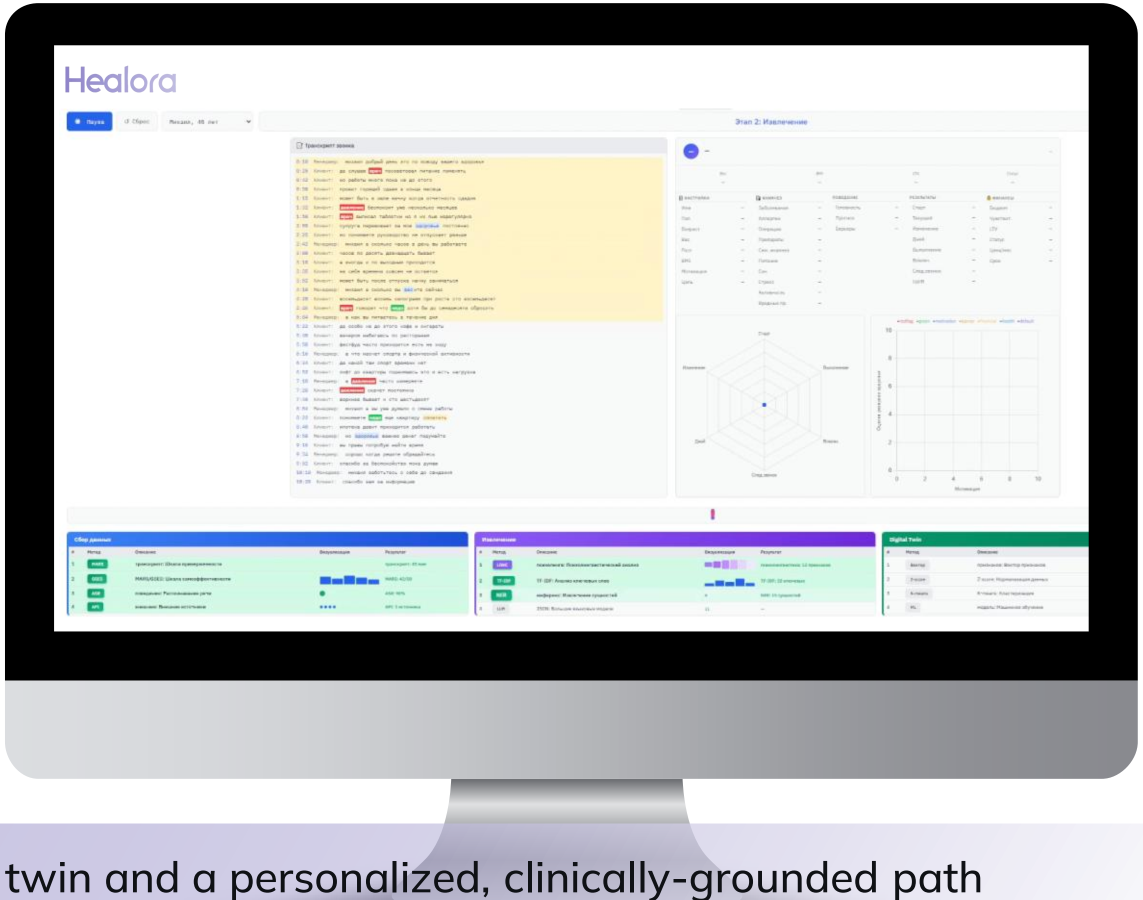
» **Clear value chain**
Labs → Service → Blockchain-based LPath → Ecosystem (clinics, nutrition, sports, mental)



See how Healora turns blood, genetics, and wearables into daily action

Demo Flow

- **Landing Page**
  value proposition
- **Data Upload**
  blood, genetic, wearable
- **Digital Twin Generation**
  visualization
- **Longevity Path**
  personalized recommendations
- **Progress Tracking**
  longitudinal monitoring

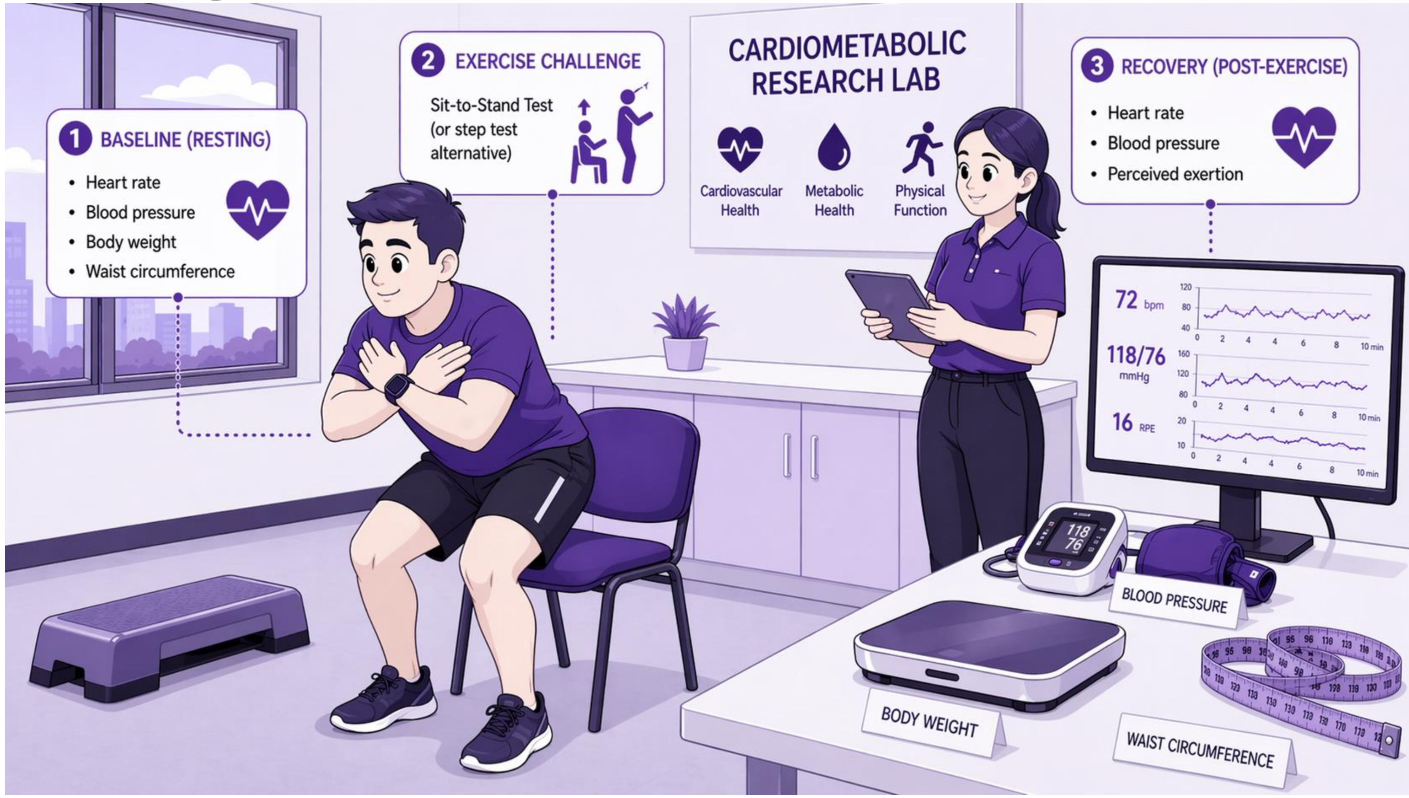


We turn fragmented health data into a living digital twin and a personalized, clinically-grounded path to 10+ additional healthy years — delivered through a messenger-native interface that users actually stick with

A simple, low-cost physiological test to initialize and update the Digital Twin

Update the Digital Twin

- **Equipment**
  wearable heart-rate sensor, chair, digital scale, waist tape, optional blood-pressure cuff, smartphone
- **Experiment protocol**
  5 min seated rest → 30-s sit-to-stand or 2-min step test → 2 min recovery → record BMI/waist/BP
- **Measured outputs**
  Resting HR, peak HR, ΔHR, heart-rate recovery at 1 and 2 min, optional HRV / SpO_2, BMI, waist circumference, optional BP
- **ML output**
  Obesity-related cardiometabolic / autonomic risk score for the Digital Twin



Goal | **Initialize/update the Digital Twin with obesity-related physiology**

Where we go:
markets, channels, audience

Channel	Use Case
Clinics (endocrinology, cardiology)	Prescribe Digital Twin as a companion to GLP-1s or lifestyle therapy
Corporate wellness	Employer-paid longevity paths for high-risk employees
Insurers	Premium discounts for users who maintain path adherence
Direct-to-consumer	Telegram-native "Health Copilot" (proven Ritm model)

Target Audience

- Women 25–45, urban, full-time work
- BMI 30–40, 2+ failed diets, emotional eating pattern
- Motivated for 5–10 kg sustainable loss – expands to full longevity

TAM

$10–20B

D2C wellness twin segment

SAM

$0.36–1.1B

Russia + international

SOM

$18–54M

5–10% share

From Outreach to Scale:
Our B2B Customer Development Roadmap

Category	Companies
Digital therapeutics / chronic care	Omada Health, Livongo, Lark Health, Vida Health
Personalized nutrition and gut health	Zest, DayTwo, Viome, Thryve, Nutrino
Genetic testing and health analytics	23andMe, InsideTracker, Chronomics, Helix, Genotek
Weight loss and habit formation	Noom, BetterMe, Fabulous, Finch, Forest
AI meal logging and calorie tracking	Foodvisor, Snapcalorie, Yazio
Russian health tech (clinics, labs, aggregators)	Lissa Health, SberHealth, Yandex.Health, Gemotest, Invitro, Helix, Genotek, ProRozhdenie, MoiMedCentr, ON Clinic, Medsi, DocDoc, Doctor Ryadom, PRO Health, FitProfile, Your Trainer, Zozhnik

Next steps

» **Outreach**
Contact 10–15 priority partners
(Omada, Lissa Health, SberHealth)

» **Pilot**
Run joint pilot with 2–3 partners
(clinics / corporate wellness)

» **Scale**
Roll out white-label API
to top 20 partners within 12 months

Pricing and Key Metrics Across All Segments

Segment	Format	Price	Key Metric
B2C	Freemium → Premium monthly	$14.99 / month	8–12% conversion
B2C	Premium annual	$119.99 / year	LTV $120–180
B2C	Genetic add-on	$29.99 (one-time)	Upsell 20%
B2B (Nutritionist)	Pro subscription	$49 / month	Saves 10h/week
B2B (Clinic small)	Clinic license	$299 / month	50 patients
B2B (Clinic large)	Clinic license	$999 / month	500 patients
B2B2C (Corporate)	Per employee	$8–12 / month	3–5x ROI
B2B2C (Insurance)	Revenue share	20–30%	Outcomes-based

3-Week Sprint Plan:
Key Tasks and Owners until Dry Run